

PELVIS & LOWER LIMB FRACTURES — Revision Table

Bold = high-yield. Classifications + key management (proximal → distal).

PELVIS & ACETABULUM			
Topic	Key points		
Pelvic ring	2 innominate + sacrum (symphysis ant, SI joints post). Blood loss 2–3L, mortality ~10%. Blood at meatus = urethral rupture → no catheter		
AP compression (open book)	Frontal collision; symphysis disruption >2cm unstable → ext fix / symphyseal plate		
Lateral compression (closed book)	Side impact; pubic rami + SI/sacral; ext fix ± plate		
Vertical shear	Fall on one leg; innominate displaced; severe/unstable; ext fix + SI screws		
Isolated avulsion	AIIS (rectus), ASIS (sartorius), pubis (adductor), ischial tub (hamstrings) → rest		
Acetabular	Anterior column (ilio-pectineal, non-WB) / posterior column (ilio-ischial, WB, post dislocation) ; ORIF if displaced; AVN/sciatic/OA		
HIP & PROXIMAL FEMUR			
Injury	Classification	Treatment	Key
Posterior hip dislocation (90%)	Thompson-Epstein I–V	Urgent reduction <6h under GA	Leg flexed/adducted/IR ; sciatic 10–20%; AVN 10% (early) → 40% (delayed)
Femoral neck (intracapsular)	Garden I–IV	<70 → CRIF (cannulated screws/DHS); >70 → hemiarthroplasty; THR if active/OA	Leg short + ER; AVN risk; NO conservative; mortality 20% at 4mo
Intertrochanteric (extracapsular)	Kyle I–IV	DHS (stable) / PFN (unstable, reverse oblique) ; always surgery	F:M 6:1; unites well, no AVN; 25–30% 1-yr mortality
Femoral shaft	Spiral/transverse/comminuted	Adults → locked IM nail ; children <5 traction+spica, 5–10 flexible nail; ext fix if compound	ATLS; fat embolism + ARDS ; 1–2L blood loss
KNEE REGION			
Injury	Key points	Treatment	
Supracondylar femur	Distal fragment extension → POPLITEAL ARTERY ; bimodal	Retrograde IM nail / locking plate / DCS; check distal pulses	
Knee dislocation	EMERGENCY; popliteal artery 40–50% , peroneal nerve 25%	Urgent reduction + monitor circulation; repair + fasciotomy	
Patella	Test active extension ; haemarthrosis + fat droplets	Undisplaced → cast; displaced transverse → tension band ; comminuted → patellectomy	
Tibial plateau	Schatzker I–VI (IV = peroneal nerve; bumper fracture); exclude compartment syndrome	I screws; II/III elevate+graft+buttress; V/VI = Span/Scan/Fix (ext fix)	
LEG & ANKLE			
Injury	Classification	Treatment	
Tibia/fibula shaft	Commonest long bone, often OPEN ; twisting (spiral) vs direct (transverse)	Closed locked IM nail (standard) ; ext fix if compound; watch compartment syndrome	
Ankle (malleolar/Pott's)	Danis-Weber A (below syndesmosis), B (at), C (above)**	Non-displaced → cast; displaced A/B + all C → ORIF lateral malleolus FIRST , syndesmotic screw if unstable. Mortise view ; non-union = medial malleolus	